

Screen Vit D, Screen 'GE7'

- Full Blood Count
- Liver Function Test
- Lipid Studies
- Renal Function Test
- Calcium
- Phosphate
- **Vitamin D Total**
- High-Sensitive CRP
- Magnesium
- Fasting Blood Sugar
- **HbA1C**
- Urine FEME

RPP RM120 RM138



Care your patient with 'GSH5'

- Full Blood Count
- Liver Function Test
- Lipid Studies
- Renal Function Test
- Calcium
- Phosphate
- Fasting Blood Sugar
- **HbA1C**
- Magnesium
- High-Sensitive CRP
- **S. Iron (→complimentary)**
- Urine Feme

Individualized with 'GSH8'

- Full Blood Count
- Liver Function Test
- Lipid Studies
- Renal Function Test
- Calcium
- Phosphate
- Fasting Blood Sugar
- **HbA1C**
- Magnesium
- High-Sensitive CRP
- **RF (→complimentary)**
- **S. Iron (→complimentary)**
- Urine Feme

Customized your package for each patient

GSH5 select any 5

RPP RM152 RM170

GSH8 Select any 8

RPP RM210 RM232

- Urine MicroAlb/Cre
- Iron Status
- Apo-B & Apo-A
- Homocysteine
- TSH
- FT4
- FT3
- HBs Ag & HBsAb
- HIV 1 & 2 (Ag & Ab)
- Anti-HAV
- RPR (reflex TPAb)
- ABO

- H pylori IgG
- AFP
- CEA
- PSA
- CA 125

For GSH8 only

- CA15-3
- CA19-9
- EBV VCA IgA

For GSH5 only

- VDRF: RPR (reflex TPAb) & RF



Lung Cancer Markers (Code: LCM, RPP: RM160 tag-on)

CYFRA 21-1 is a key tumor marker for non-small cell lung cancer (NSCLC), particularly squamous cell carcinoma (SCC). It is also used in monitoring bladder cancer. This marker reflects tumor cell breakdown, making it useful for prognosis, treatment monitoring and relapse risk.¹

	Sensitivity ²	Specificity ²
Overall NSCLC	60 %	90%
Squamous Cell Carcinoma (SCC)	72 %	94%
Lung Adenocarcinoma (LAD)	48 %	94%
Large Cell Carcinoma (LCC)	67 %	93%

Reference:

1. Molina, R., et al. (2008). "CYFRA 21-1 as a Prognostic and Follow-Up Marker in Non-Small Cell Lung Cancer." *Lung Cancer*, 60(1), 67-73.
2. Lei Fu., et al. (2019). CYFRA 21-1 tests in the diagnosis of non-small cell lung cancer: A meta-analysis.

NSE is a crucial biomarker for neuroendocrine tumors, particularly small cell lung cancer (SCLC) and neuroblastoma. It is useful for both initial diagnosis and tracking treatment response.¹ NSE sensitivity for SCLC ≈ 70% (range 60–80%)

Reference:

1. Carney, D. N., et al. (1982). "Neuron-Specific Enolase: A Biomarker for Small Cell Lung Cancer." *New England Journal of Medicine*, 306(8), 409-410.
2. Huang S et al. Diagnostic value of neuron-specific enolase for small cell lung cancer: a meta-analysis. *Tumour Biol.* 2015;36(5):3373–3380. (Meta-analysis, 32 studies)



CA72-4 (Code: 724, RPP: RM80 tag-on)

CA72-4 is a well-recognized biomarker for gastrointestinal malignancies, particularly gastric cancer. It is also used in monitoring ovarian and colorectal cancers. Elevated levels may indicate the presence of malignancies, making it a valuable tool for diagnosis, evaluating treatment response and recurrence.¹

CA72-4 has shown promise as a tumour marker for gastric cancer. In a study comparing CA72-4 with CEA and CA19-9, at a specificity of 95%, the sensitivities were²:

CA72-4: 94% CEA: 25% CA19-9: 41%

Note: CA72-4 was specifically upregulated in patients with gouty arthritis, high level was the strongest predictor of gout flares.³

Reference:

1. Stieber, P., et al. (2003). Serum CA 72-4 in the Diagnosis and Follow-Up of Gastric Cancer." *Cancer*, 98(5), 907-913.
2. D J Bynae et al. *Br J Surg.* (1990 Sep). CA72-4: a new tumour marker for gastric cancer.
3. *Rheumatology (Oxford)*. 2020 Feb 22;59(10):2872–2880. doi: 10.1093/rheumatology/keaa046